



EVERGREEN COMMUNITY CARE INC.

BEHAVIOUR SUPPORT

POLICY AND PROCEDURE MANUAL

Positive, Trauma-Informed, and Rights-Based Behaviour Support for All Residential Care Programs

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Replaces	Behaviour Management sections of RC Program Manual v3 and v4
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Applies To	All staff, students, and volunteers at all Evergreen Community Care residential care programs
MCFD Standards	Standard C.1 (Personal Safety); C.3 (Physical Restraint); D.7 (Behaviour Management); D.8 (Decision-Making Participation); G.3 (Staff Training); G.4 (Staff Supervision) — SHSS April 2026
Legislative Authority	CFCSA s.70, s.71; Residential Care Regulation (RCR) s.42–44; BC Human Rights Code; MCFD Policy 5.13 (Physical Restraint); DRIPA (SBC 2019, c.44)
Related Documents	RC Program Manual v4 (ECC-RC-POL-2026-003); HR Policy Binder v3; OHS Program Binder; Youth Worker Orientation Manual; PC Manual; PM Manual; QA Program Binder

This is a new standalone manual. It consolidates and expands all behaviour support content previously distributed across the RC Program Manual, PC Manual, PM Manual, and OHS Program Binder. All staff must read this manual in full and sign the acknowledgement at Appendix H before their first shift with youth.



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PART 1 — Foundation: Philosophy, Rights, and Legislative Framework

1. POLICY STATEMENT AND GUIDING PHILOSOPHY

Evergreen Community Care Inc. is committed to providing behaviour support that is positive, trauma-informed, strengths-based, and fully consistent with every child and youth's legal rights and dignity. Every behaviour has meaning. Behaviour is communication — most often the communication of unmet need, past harm, fear, or disconnection. Our role is to understand and respond to that communication, not to suppress or punish it.

This manual governs all behaviour support practices across every Evergreen Community Care residential care program. It establishes what is required, what is permitted, what is prohibited, and what happens when these standards are not met.

1.1 Core Principles

Principle	What It Means in Practice
Safety first	The physical and emotional safety of every child and youth — and every staff member — is always the first priority. Every behaviour support decision is evaluated against this principle.
Behaviour is communication	Challenging behaviour is most often a response to unmet need, trauma triggers, developmental stage, or relational rupture. We ask 'what is this behaviour telling us?' before deciding how to respond.
Trauma-informed practice	Children and youth in residential care have experienced significant trauma. Their nervous systems respond to perceived threat in ways that can look like 'behaviour problems.' We understand and respond to the trauma beneath the behaviour.
Least intrusive intervention	We always use the least intrusive, least restrictive intervention that addresses the immediate safety concern. More intrusive responses are used only when less intrusive options have been tried or are clearly insufficient.
Dignity and respect	Every intervention preserves the youth's dignity. We never humiliate, demean, threaten, or publicly shame. A youth in crisis is a person in pain — not a problem to be managed.
Genuine relationships	Behaviour support is most effective within a genuine caring relationship. Staff invest in knowing each youth as a whole person — their history, strengths, fears, cultural identity, and aspirations.
Youth voice	Youth are meaningfully involved in developing their own Behaviour Support Plans. Their input is documented and reflected in the plan —



	not as a token signature but as a record of genuine participation.
Cultural and identity safety	Every youth's cultural identity, Indigenous heritage, gender identity, and sexual orientation are affirmed and respected in all behaviour support decisions. These identities are never treated as behaviour problems.
Evidence-based approaches	Behaviour support strategies are grounded in peer-reviewed evidence: trauma-informed care; attachment theory; positive behaviour support; non-violent crisis intervention; and strengths-based practice.

2. LEGISLATIVE AND STANDARDS FRAMEWORK

Authority	Relevance to Behaviour Support	Key Provisions
Child, Family and Community Service Act (CFCSA)	Primary legislation; establishes rights framework for all children in care	s.2 (principles — paramourcy of safety); s.70 (14 rights of children in care including freedom from corporal punishment, reasonable privacy, and right to complain freely); s.14 (mandatory reporting)
Residential Care Regulation (RCR), amended April 2026	Specific operational requirements for residential care facilities	s.42 (care and supervision); s.43 (behaviour support requirements); s.44 (prohibited practices — specific list)
MCFD Standards — SHSS, April 2026	MCFD operational standards for all contracted residential care providers	Standard C.1 (personal safety); C.3 (physical restraint — threshold, conditions, documentation); D.7 (positive practices; prohibited practices); D.8 (decision-making participation); G.3 (mandatory training including NVCI, trauma-informed, gender-affirming); G.4 (supervision including gender-affirming care — G.4.3(c) mandatory April 2024)
MCFD Policy 5.13 — Physical Restraint	MCFD's specific operational policy on physical restraint in residential care	Threshold requirements; prohibited methods (prone restraint absolutely prohibited); documentation; reporting; MCFD notification



		requirements; post-restraint obligations
BC Human Rights Code (RSBC 1996, c.210)	Prohibits discrimination in all behaviour support decisions	Protected characteristics include race, colour, ancestry, religion, sex, sexual orientation, gender identity or expression, disability — none may be a basis for more restrictive behaviour support
Declaration on the Rights of Indigenous Peoples Act (DRIPA, SBC 2019)	Implements UNDRIP in BC; shapes culturally responsive behaviour support for Indigenous youth	Articles 7, 8, and 14 of UNDRIP — right to maintain cultural identity; right to education in culture and language; right to dignity
Workers Compensation Act and OHS Regulation	Workplace safety obligations — staff have the right to work safely	OHS Reg. s.4.27–4.31 (workplace violence risk assessment and prevention); all behaviour support practices must protect staff safety

3. RIGHTS OF CHILDREN AND YOUTH IN CARE

Every child and youth in an Evergreen Community Care residential care program retains all of the rights established in CFCSA s.70. Behaviour support decisions must be consistent with these rights at all times. The rights are not suspended in response to any behaviour.

CFCSA s.70 Right	Application to Behaviour Support	Violations to Prevent
Freedom from corporal punishment	No physical force is ever used as a consequence or deterrent — only as an emergency protective measure under the strict physical restraint policy (Section 14)	Spanking, grabbing, pushing, or any physical contact intended to cause pain, discomfort, or fear as a consequence
Freedom from cruel, unusual, or degrading treatment	No humiliation, public shaming, degrading consequences, or demeaning language in any behaviour support response	Name-calling; mocking; contemptuous remarks; deliberately embarrassing a youth in front of peers
Reasonable privacy	Privacy maintained during personal care, room checks, and all behaviour support responses	Unnecessary observation; discussing a youth's behaviour history in front of peers or visitors



Reasonable access to family, extended family, and communities	Family contact is never withheld as a consequence for any reason — including behaviour	Restricting phone calls or visits as punishment for behaviour
Know the standard of behaviour expected and consequences if not met	BSP clearly documents expected behaviours and logical consequences in language the youth understands; communicated at admission and updated at every review	Applying consequences the youth was never informed of in advance
Be informed of and have a say in significant decisions	Youth meaningfully participate in BSP development; input is documented	Writing a BSP without the youth's genuine input or informed understanding
Receive services respecting dignity, culture, and identity	Behaviour support responds to the whole person — trauma history, cultural heritage, gender identity, developmental needs	One-size-fits-all consequence systems without individualization

⚠️ ABSOLUTE RULE: No right listed in CFCSA s.70 is ever suspended or withheld as a behaviour consequence. Any attempt to use these rights as leverage (e.g., 'you won't be able to call your family unless you...') is a rights violation and a Critical Incident requiring MCFD notification same day.

4. ROLES AND RESPONSIBILITIES

Role	Behaviour Support Responsibilities
Youth Care Worker (YCW)	Implement BSP strategies consistently across all shifts; complete shift log entries for all significant behaviour incidents; de-escalate using approved strategies; use physical restraint only when the legal threshold is met and only with current NVCI certification; document all restraint uses on the same shift; never use any prohibited practice under any circumstances; report any observed prohibited practice by a colleague immediately to supervisor
Shift Lead	All YCW responsibilities plus: ensure BSP is reviewed with incoming staff at handover during high-risk periods; coordinate backup for de-escalation and restraint situations; complete or supervise all incident documentation before end of shift; notify PC of any restraint on same shift
Program Coordinator (PC)	Receive and review all behaviour incident reports within 24 hours; verify all restraint documentation within the shift it occurred; initiate BSP review following any Category 1 CI; track restraint incidents monthly and report trends to PM; conduct prohibited practices review with team minimum annually; ensure all staff have current NVCI certification before any shift where restraint may be required; co-



	develop all BSPs in consultation with the youth and MCFD social worker; implement PM-directed corrective actions
Program Manager (PM)	Approve all Behaviour Support Plans before MCFD submission; personally review every physical restraint incident within 24 hours; initiate threshold review when 3+ restraints involving the same youth in 30 days; notify MCFD social worker of any restraint use within 24 hours; submit CI Reports for all Category 1 behaviour incidents; ensure no staff member performs physical restraint without current NVCI certification; maintain the restraint register; conduct annual violence risk assessment including behaviour-related risks; present quarterly behaviour and restraint trend data to ED
Executive Director (ED)	Approve this manual annually; receive PM quarterly behaviour and restraint trend report; notify MCFD of any pattern of restraint or prohibited practice that rises to the organizational level; ensure all programs are adequately resourced for behaviour support — staffing ratios, NVCI training, BSP development time
MCFD Social Worker	Participates in BSP development for their caseload youth; notified of all Category 1 CIs including restraints within 2 hours; may direct additional assessment or service coordination if behaviour concerns indicate unmet need; approves any planned physical intervention if required by MCFD Policy 5.13



PART 2 — Behaviour Support Plans

5. BEHAVIOUR SUPPORT PLAN (BSP) — REQUIREMENTS AND DEVELOPMENT

Every child and youth placed at an Evergreen Community Care residential program must have an individualized Behaviour Support Plan (BSP). The BSP is a living document — it is the primary tool for ensuring all staff understand the youth's behaviour patterns, respond consistently, and implement supports that build the youth's own capacity to regulate, communicate, and thrive.

5.1 When a BSP Is Required

Trigger	Timeline
All admissions — every youth	Preliminary BSP within 48 hours; full BSP within 14 days
Youth with documented behaviour support needs in transfer documentation	Full BSP at admission; consult with transferring provider before placement begins
Following any Category 1 CI involving behaviour	BSP review and update within 5 business days
Following 3 or more incidents in 30 days involving the same behaviour	Threshold review — PM initiates BSP revision meeting within 5 business days
Upon significant change in circumstances (new diagnosis, trauma disclosure, ISP change)	BSP updated within 10 business days
Every 90 days — minimum	BSP review at every ISP review; documented even if no changes made

5.2 BSP Development Process

Step	Action	Responsible
1	Gather information — review all available records: prior BSPs, assessment reports, MCFD file summary, school records, youth's self-report of what helps and what doesn't	PC + PM



2	Meet with the youth — conduct a genuine, conversational BSP development session; ask: what makes things hard? What helps when things get hard? What do you want staff to do? What do you not want?	PC (leads); youth attends
3	Consult the team — gather input from all direct-care staff who know the youth; document patterns and effective strategies they have observed	PC
4	Consult MCFD social worker and family where appropriate — may have assessment information or context about what works	PC + PM
5	Draft the BSP using the template (Appendix A) — every required component completed; no blank fields	PC
6	Review draft with the youth — confirm it accurately reflects their input; make changes if they disagree; document youth's response	PC + youth
7	PM reviews and approves BSP before MCFD submission	PM
8	Distribute to all staff — every staff member working with this youth reads and signs the BSP Handover Checklist (Appendix G) before their first solo shift	PC
9	File in youth's ISR file and MCFD record management system	PC

6. BSP COMPONENTS AND DOCUMENTATION STANDARDS

Every Behaviour Support Plan must contain all of the following components. The PC is responsible for completeness; the PM approves before MCFD submission. Incomplete BSPs are returned within 48 hours.

BSP Component	Required Content
1. Youth Information	Legal name; preferred name; pronouns; date of birth; cultural/Nation affiliation; program; room; PC; PM; MCFD social worker; BSP date; review date
2. Youth Strengths and Interests	Genuine strengths — at least 5; interests; important relationships; cultural connections and sources of pride. This section comes first



	because it shapes how we understand everything else.
3. Background and Behaviour History	Relevant trauma history (broad terms); prior diagnoses relevant to behaviour support; prior BSPs — what worked and what did not; known ineffective responses from prior placements
4. Current Behaviour Patterns	Specific behaviours addressed; frequency, intensity, duration (FID) baseline; ABC analysis summary (Antecedent — Behaviour — Consequence); function of behaviour (what need is it communicating?)
5. Triggers and Warning Signs	Identified antecedents and triggers; early warning signs at each crisis level — specific to this youth
6. De-escalation Strategies — individualized	Specific strategies that WORK for this youth; strategies that DO NOT WORK — staff must know what to avoid; sensory and environmental modifications that help
7. Approved Behaviour Support Strategies	Specific positive reinforcement and teaching strategies; logical consequences that are pre-approved and proportionate; schedule and routine supports; environmental modifications
8. Crisis Response Plan	Step-by-step response for this youth at each crisis level; contact sequence; when to call 911; when to contact MCFD; specific language that helps in crisis; specific things to avoid
9. Physical Restraint Authorization	Whether restraint is authorized; specific authorized NVCI holds; holds contraindicated due to medical or trauma history; MCFD social worker authorization if required by Policy 5.13
10. Post-Incident Protocol	What this youth needs after a crisis or restraint; preferred debrief approach; staff the youth is most comfortable with post-incident
11. Skill-Building Goal	At least one measurable goal: what capacity is being built so this youth needs less restrictive support over time? Timeline and measurement method.
12. Consent and Participation	Youth's signature or documented verbal participation; youth's areas of agreement and disagreement; parent/guardian signature if applicable; MCFD social worker acknowledgement
13. Review History	Date of each review; summary of changes; participants; next scheduled review date

7. BSP REVIEW, UPDATE, AND COMMUNICATION

7.1 Review Schedule

Review Trigger	Required Response
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Every 90 days (minimum — concurrent with ISP review)	Full review; document whether changes made and why; PM approves revised BSP; redistribute to all staff within 24 hours
Category 1 CI involving behaviour	Review within 5 business days; update strategies and crisis plan as needed
3+ incidents involving the same behaviour in 30 days	Threshold review — PM chairs; examine BSP effectiveness; consult MCFD social worker
New diagnosis, medication change, or significant life event	Review within 10 business days
Youth requests a review	Review within 10 business days — youth's request is taken seriously
At transition or discharge	Final BSP update; transfer summary documents current strategies; distributed to receiving provider

7.2 Shift Communication Requirements

- Every new staff member — permanent, casual, or relief — reads and signs the BSP Communication Checklist (Appendix G) before their first solo shift with any youth
- At every shift handover, the outgoing shift lead verbally communicates current behaviour status of every youth, including any recent trigger events or de-escalation strategies in active use
- During any period of heightened risk or active crisis, the BSP is reviewed at the start of every shift
- Updated BSPs are redistributed immediately — all previous versions removed from the shift communication area; staff sign the updated checklist within 24 hours of the update
- The current BSP for every youth is accessible in the shift communication binder at all times

PART 3 — Positive Behaviour Support Practices

8. TRAUMA-INFORMED AND ATTACHMENT-BASED PRACTICE

The majority of children and youth in residential care have experienced significant developmental trauma — abuse, neglect, family violence, removal from family, multiple placements, and related adverse experiences. Understanding this context is not an excuse for behaviour — it is the foundation for understanding behaviour and building effective support.

8.1 What Trauma Does to Behaviour

What We See	What Is Actually Happening
Defiance, refusal, non-compliance	Often a protective response — the youth has learned that compliance leads to harm; control over small things feels essential to survival
Aggression — verbal or physical	Threat response activated by perceived danger; the nervous system cannot distinguish past threat from present threat; sometimes the only language a youth has for overwhelming distress
Emotional dysregulation — intensity disproportionate to trigger	Developmental gaps in emotional regulation; trauma has interrupted the co-regulation experiences needed to build self-regulation capacity
Dissociation — 'checked out,' unresponsive	Freeze response to overwhelming stress; not defiance; the youth is not ignoring staff — they are not fully present
Sexualized behaviour	Most often reflects exposure to inappropriate sexual contact or content; requires a therapeutic response, not a punitive one
Self-harm	Often a coping mechanism for overwhelming emotional pain; sometimes the only way a youth knows to feel in control; requires safety intervention and therapeutic support, not discipline
Lying and what appears to be manipulation	Often survival strategies learned in environments where honesty was unsafe; reflects past experience, not character
Extreme attachment behaviour — clinginess or rejection	Attachment injuries from early caregiving; testing whether this relationship is safe; approach-avoid pattern is normal for traumatized youth

8.2 Trauma-Informed Responses

- Regulate yourself first — your nervous system regulates theirs. A calm, grounded staff presence is the most powerful de-escalation tool available.



- Name and validate the feeling before addressing the behaviour: 'I can see you're really upset right now. That makes sense.'
- Avoid power struggles — offer limited genuine choices where possible; power struggles escalate; choice reduces the threat response
- Use 'with' language not 'at' language — 'Let's figure this out together' not 'You need to...'
- Separate the behaviour from the person — 'That behaviour isn't okay' not 'You're a bad person'
- Repair after rupture — every challenging interaction is an opportunity for relationship repair; come back after the crisis and reconnect
- Maintain connection — youth in crisis most need relationship; the moment of crisis is not the moment to withdraw
- Think long-term — the goal is not compliance in this moment; it is building the youth's capacity for self-regulation over time

9. APPROVED BEHAVIOUR SUPPORT STRATEGIES

9.1 Positive Reinforcement

Strategy	Description and Application
Specific verbal praise	Immediate, specific, genuine acknowledgement of positive behaviour: 'I noticed you walked away from that situation instead of arguing — that took real strength.' Generic praise ('good job') is less effective.
Natural reinforcers	Activities, privileges, and social recognition the youth has identified as meaningful. Documented in the BSP. Never withheld as punishment for unrelated behaviour.
Point or token systems (individualized)	Only where documented in the BSP; youth understands and agreed to the system; never applied punitively; never used to withhold basic needs or CFCSA s.70 rights
Relationship-based reinforcement	Time, attention, conversation, shared activities with staff the youth trusts. The most powerful positive reinforcer available in residential care.
Progress recognition	Explicitly naming growth — even small growth: 'Six months ago, this would have been a really hard situation for you. Look at how you handled it today.'

9.2 Environmental and Structural Supports

- Predictable, consistent daily schedule reduces anxiety and behaviour — post the schedule visibly and explain any changes in advance
- Sensory environment modifications — lighting, sound level, access to quiet space — documented in BSP and implemented consistently



- Transition warnings — five-minute and two-minute warnings before any schedule change or activity transition
- Designated calm-down spaces — youth-identified, not imposed; going to a calm space is never a punishment
- Reduced demands during known high-risk times — mornings, mealtimes, return from school, evenings
- Consistent staffing — relationship continuity is a structural intervention; minimize changes in assigned staff where possible

9.3 Logical Consequences

Logical consequences are an approved behaviour support tool when they meet all of the following criteria:

Criterion	Requirement
Pre-disclosed	The youth was informed of the consequence before the behaviour occurred; documented in the BSP
Directly related	The consequence is logically connected to the behaviour (e.g., if you damage shared property, you contribute to repairing it)
Proportionate	The severity of the consequence matches the severity of the behaviour
Respectful	Delivered calmly and without humiliation, anger, or contempt
Does not violate rights	Does not restrict CFCSA s.70 rights; does not withhold meals, basic care, or family contact
Consistent	Applied consistently across all staff and all shifts for the same behaviour

10. DE-ESCALATION TECHNIQUES AND CRISIS PREVENTION

De-escalation is always the first response to any escalating situation. Physical intervention is a last resort used only when de-escalation has not prevented imminent serious harm. The following techniques are approved and should be practised until they are automatic.

10.1 Universal De-escalation Principles

Principle	Application
REGULATE YOURSELF FIRST	Your physiological state directly affects the youth's state. Slow your breathing, lower your voice, relax your posture. If you feel flooded, request backup and step back.



Reduce stimulation	Lower your voice volume and pitch; reduce environmental noise; create physical space; reduce number of people in the immediate area
Maintain respectful distance	Respect personal space; never stand over a youth who is on the floor or seated; get to their level where safe
Use simple, clear language	Short sentences; one instruction at a time; no threats; no ultimatums; avoid 'you need to' — use 'when you're ready'
Acknowledge feelings before behaviour	'I can see this is really hard right now.' Naming the emotion before addressing the behaviour reduces the threat response.
Offer genuine choices	Two real options; both are acceptable to you; this restores a sense of control
Avoid power struggles	Do not escalate demands; withdraw unnecessary directives; the goal is safety, not compliance
Call for backup early	Before the situation escalates; being two people removes the requirement for physical intervention in most situations
Give space when safe	Sometimes the most effective de-escalation is strategic withdrawal — maintaining visual contact from a greater distance
Time	De-escalation often requires time. Do not rush. A youth in the stress response cannot hear complex instructions.

⚠ Staff are NEVER required to physically manage a behavioural crisis alone. If you are the sole staff member on shift and the situation requires backup, call the on-call supervisor immediately and follow the Emergency Staffing Protocol. Do not attempt physical restraint alone.

11. GENDER-AFFIRMING AND CULTURALLY SAFE BEHAVIOUR SUPPORT

11.1 Gender Identity and Expression

MCFD Standards D.1.13(b), D.6(g), G.3.2(b), and G.4.3(c) were all amended in April 2024 to explicitly require gender-affirming care. Gender identity and expression are never behaviour problems. The following requirements are mandatory for all staff:

- Use every youth's correct name and pronouns consistently and from Day 1. There is no adjustment period. Deliberate misgendering is a Code of Ethics violation subject to disciplinary action.
- Never use gender non-conforming expression or identity as a rationale for a more restrictive behaviour support response. A youth's clothing choices, name, pronouns, or gender expression are not behaviour issues.



- Emotional distress related to gender identity or dysphoria is supported with affirmation and connection — not behaviour management consequences.
- Gender identity is confidential. Never disclose a youth's gender identity, deadname, or pronouns to any person — including family members — without the youth's informed consent.
- Physical environment supports gender expression: access to clothing, hygiene products, and space that affirms the youth's gender identity.

11.2 Cultural Safety for Indigenous Youth

- Cultural practices — smudging, ceremony, drumming, traditional foods, language, regalia — are never restricted as behaviour consequences and are actively facilitated
- Behaviour that reflects cultural expression or identity is never pathologized or treated as a behaviour problem
- The BSP includes cultural context where the youth or family have shared it; consults the Nation or DAA where appropriate
- Racial slurs, stereotyping, or any racially discriminatory comment directed at a youth — by any staff member — is a Code of Ethics violation and a Category 1 Critical Incident
- Indigenous Cultural Safety and Humility training (San'yas or FNHA equivalent) is mandatory for all staff within 60 days of hire and annually

PART 4 — Prohibited Practices

12. COMPLETE LIST OF ABSOLUTELY PROHIBITED PRACTICES

The following practices are absolutely prohibited under all circumstances — regardless of the behaviour, regardless of the instruction of any individual including supervisors, and regardless of any claimed emergency. Any practice on this list constitutes abuse or neglect under the CFCSA and triggers mandatory reporting obligations under CFCSA s.14.

⚠ If any person — including a supervisor or manager — instructs you to use a prohibited practice, you REFUSE, document the instruction, and report it to the next level of management or the ED immediately. Following an instruction to use a prohibited practice is not a defence. Every staff member is individually accountable.

Prohibited Practice	Authority	Notes
Corporal punishment — any physical contact intended to cause pain, discomfort, or fear as a consequence	CFCSA s.70; RCR s.44(a); MCFD SHSS D.7	Includes spanking, slapping, pinching, flicking, forced postures, deliberately rough handling. Absolutely prohibited — no exception.
Seclusion — locking or confining a youth alone in a room against their will	RCR s.44(b); MCFD SHSS D.7; MCFD Policy 5.13	Includes locking in bedroom, bathroom, any room; blocking exit with physical presence; preventing youth from leaving a space. 'Quiet time' in an unlocked space the youth can voluntarily leave is NOT seclusion.
Withholding meals or snacks as a consequence	CFCSA s.70; RCR s.44(c); MCFD SHSS D.7	Every youth receives 3 meals and 2 snacks daily — no exceptions. Meal content may not be modified punitively.
Withholding family, community, or cultural connection as a consequence	CFCSA s.70; RCR s.44; MCFD SHSS D.7	Family contact is a legal right — not a privilege. Phone access to family cannot be restricted as a behaviour consequence.
Withholding basic personal hygiene items or	RCR s.44(d); MCFD SHSS D.7	Includes soap, toothbrush, shower access, clean clothing. Non-negotiable.



opportunities		
Psychologically demeaning or humiliating treatment	CFCSA s.70; MCFD SHSS D.7	Includes public shaming; contemptuous language; mockery; name-calling; deliberately embarrassing a youth in front of peers; cruel or degrading comments
Threats — of harm, placement change, or involving police or MCFD as a behaviour consequence	CFCSA s.70; MCFD SHSS D.7	Threatening to call police, MCFD, or to remove a youth from the placement as a behaviour management response is prohibited. These may only occur when legally required.
Prone restraint — face-down physical restraint in any form	MCFD Policy 5.13; MCFD SHSS C.3	Absolutely prohibited. Risk of positional asphyxia. Zero exceptions. Any prone restraint requires immediate MCFD notification.
Restraint applying pressure to the chest, neck, or head	MCFD Policy 5.13; MCFD SHSS C.3	Risk of asphyxia and fatal injury. Zero exceptions under any circumstances.
Restraint as punishment, deterrent, or for convenience	MCFD Policy 5.13; MCFD SHSS C.3	Physical restraint may only be used as an emergency protective measure when imminent serious physical harm is the only alternative.
Body or strip searches of any youth	CFCSA s.70; MCFD SHSS B.3	Absolutely prohibited. No staff member has authority to search a youth's body or require removal of clothing.
Withholding prescribed medication as a behaviour consequence	RCR s.45–49; MCFD SHSS D.7	All prescribed medications are administered as prescribed regardless of behaviour.
Denying school attendance as a behaviour consequence	School Act; MCFD SHSS D.5	School attendance is a legal right. Youth may only be suspended by school administration through the proper process.
Denying access to the Representative for Children and	CFCSA s.70; Advocate for Children and Youth Act	Youth have unrestricted access to the RCY (1-800-476-3933) at any time, in



Youth or Ombudsperson		private. Blocking this access is a rights violation.
Financial consequences not authorized in the BSP	CFCSA s.70; Finance Manual s.14	Youth allowances are never withheld as punishment. Any financial consequence must be pre-authorized in the BSP and proportionate.
Peer-administered consequences or group punishment for one youth's behaviour	MCFD SHSS D.7	Youth are never tasked with enforcing rules on other youth; group-based punishments for one youth's behaviour are prohibited.
Deliberately delaying or denying medical attention as a consequence	CFCSA s.70; RCR	Youth in medical need receive care immediately regardless of behaviour.
Recording or photographing youth without MCFD authorization	CFCSA s.70; FOIPPA	No staff member photographs, videos, or records any youth for any purpose without written MCFD authorization in the youth's file.

13. CONSEQUENCES OF PROHIBITED PRACTICE USE

13.1 Immediate Response Protocol

Step	Action	Responsible
1	If you witness a prohibited practice: intervene immediately and calmly to stop it; ensure the youth is safe; separate the involved staff member from the youth	Witnessing staff member
2	Notify PC or on-call supervisor immediately — same shift	First staff member aware
3	PC notifies PM within 2 hours	PC
4	PM notifies ED same day	PM
5	PM notifies MCFD social worker same day — this is a Category 1 Critical Incident	PM
6	Involved staff member placed on suspension	PM (implements); ED



	with pay pending investigation	(authorizes)
7	CI Report completed within 24 hours	PM
8	Youth receives a check-in from a trusted staff member; their wellbeing is the immediate priority; any disclosure treated as potential abuse disclosure under CFCSA s.14	PC or designated staff

13.2 Investigation and Discipline

- Every alleged or confirmed prohibited practice is investigated by the PM within 24 hours, with ED involvement from the outset
- The investigation is documented in writing; the involved staff member is given an opportunity to respond
- If the prohibited practice constitutes abuse or neglect under the CFCSA, the Mandatory Reporting obligation under CFCSA s.14 is triggered — the PM or ED reports to MCFD within 24 hours
- If the prohibited practice constitutes a criminal act, police are notified
- Summary dismissal (immediate termination without notice) applies to: any confirmed use of corporal punishment; any confirmed use of prone restraint; any confirmed seclusion; any confirmed withholding of medical care; any confirmed body search
- Progressive discipline may apply to less severe prohibited practice violations at ED and HR discretion

13.3 Mandatory Reporting Obligation — CFCSA s.14

Every staff member who has reason to believe that a child or youth has been or is likely to be physically harmed, sexually abused or exploited, or emotionally harmed as a result of a prohibited practice by any person (including a colleague) has a personal, non-delegatable mandatory reporting obligation under CFCSA s.14. This obligation cannot be satisfied by reporting to your supervisor. If you have reasonable grounds to believe a child has been harmed, you must report to MCFD. Your supervisor cannot tell you not to. Failure to report is an offence under the CFCSA.

MCFD Child Protection Intake: 1-800-663-9122 (24/7 Provincial Centralized Screening). You may report anonymously. Report first; document that you reported; then notify your supervisor.

PART 5 — Physical Restraint

14. PHYSICAL RESTRAINT POLICY

Physical restraint is an emergency protective measure of absolute last resort. It is never a behaviour management tool, a consequence, a deterrent, or a routine intervention. Every use of physical restraint at Evergreen Community Care is a serious event that is documented, reviewed, and reported to MCFD. The organizational goal is a program where physical restraint is never needed — and every use is an opportunity to learn what more is needed to achieve that goal.

14.1 Legal Threshold — All Four Conditions Must Be Simultaneously Met

Condition	Requirement
1. Imminent serious physical harm	There is an immediate and serious risk of serious physical harm to the youth or another person. Imminent means the harm is about to occur. Serious means significant injury — not discomfort or property damage.
2. De-escalation has failed or is clearly insufficient	All reasonable de-escalation attempts have been made and have not prevented the imminent harm. In an acute crisis where there is literally no time, this condition is met by the acuity of the situation.
3. NVCI certification current	The staff member using physical restraint has a current non-expired NVCI certification. An uncertified staff member never uses physical restraint. Ever.
4. Least restrictive method	The restraint method used applies the minimum force necessary for the minimum time necessary to prevent the immediate harm.

⚠ Physical restraint is NEVER used for: compliance; punishment; convenience; deterrence; property damage alone; verbal aggression alone; or any behaviour not meeting the imminent serious physical harm threshold. Using restraint outside this threshold constitutes assault.

14.2 Who May Use Physical Restraint

Requirement	Detail
NVCI certification — mandatory	Current NVCI certification required before any staff member may use physical restraint. NVCI Module 1 theoretical only is not sufficient to authorize restraint use.
Two-person requirement	Where possible, a second staff member must be present: monitors breathing and colour; provides verbal de-escalation; calls 911 if indicated; manages the environment. Document solo restraint



	circumstances explicitly.
Medical contraindications	Check the youth's BSP Section 9 for any contraindicated holds due to medical history. These must be observed absolutely.
No solo restraint policy	No staff member uses physical restraint alone where it is avoidable. Staff safety, youth safety, and documentation integrity all require a second person.

15. AUTHORIZED RESTRAINT METHODS (NVCi)

The only authorized restraint methods at Evergreen Community Care are those taught in the current Non-Violent Crisis Intervention (NVCi) curriculum from the Crisis Prevention Institute. Any hold or technique not taught in NVCi is not authorized — regardless of the staff member's other training or experience.

15.1 Authorized Methods

- Team control position (standing escort) — authorized
- Side restraint position — authorized
- Two-person transport positions as taught in current NVCi curriculum — authorized
- Therapeutic holding techniques as taught in current NVCi curriculum — authorized

15.2 Absolutely Prohibited Restraint Methods — No Exception

Prohibited Method	Reason
Prone restraint — face-down in any form	Risk of positional asphyxia and sudden death; prohibited by MCFD Policy 5.13; zero exceptions
Any pressure on the chest, sternum, or abdomen	Risk of asphyxia; prohibited under all circumstances
Any pressure on the neck, throat, or airway	Risk of asphyxia and fatal injury; zero exceptions
Any pressure on the head or face	Risk of injury and aspiration; prohibited
Joint locks or pain compliance techniques	Not NVCi; never authorized at Evergreen Community Care
Chokehold or any hold restricting the airway	Risk of fatal asphyxia; prohibited by MCFD Policy 5.13 and criminal law
Any technique not in the	Not authorized; not covered by policy or training



current NVCI curriculum	
Restraint by a staff member with expired or no NVCI certification	Not authorized under any circumstances

16. PHYSICAL RESTRAINT PROCEDURES — STEP BY STEP

Step	Action	Notes
1. ATTEMPT DE-ESCALATION	Use all available de-escalation strategies before any physical intervention. Call for backup.	Even in rapidly escalating situations, verbal de-escalation takes priority. Document all attempts.
2. CALL FOR BACKUP	Call for the second staff member before physical intervention begins where any time is available.	Two-person restraint is the standard. Solo restraint is a last resort documented fully.
3. VERBAL WARNING	Give a calm, clear verbal direction: '[Name], I need you to stop. I don't want to have to hold you. Please [specific direction].'	The verbal direction must be audible, specific, and calm. Document the exact words used.
4. APPLY MINIMUM RESTRAINT	If the imminent harm threshold is met and verbal direction has not worked: apply the least restrictive authorized NVCI hold.	Only apply what is necessary. Minimum force for minimum time.
5. MONITOR BREATHING AND COLOUR	Second staff member continuously monitors: breathing rate and rhythm; skin colour; level of consciousness; distress level.	If breathing becomes laboured, colour changes, or youth becomes unresponsive: RELEASE IMMEDIATELY and call 911.
6. CONTINUE DE-ESCALATION	Maintain calm verbal de-escalation throughout the restraint. Do not lecture, threaten, or moralize.	'I'm going to hold you until it's safe. When you're ready, I'll let you go. You're safe.'
7. RELEASE AS SOON AS SAFE	Release as soon as the imminent harm has passed — even if the youth is still upset.	Document the time of restraint initiation and release. Every minute counts.
8. ENSURE SAFETY POST-RELEASE	Verify the youth is physically unharmed. Offer water and a calm space. Call 911 if any injury is evident or reported.	Injuries sustained during restraint require medical attention immediately.
9. NOTIFY SUPERVISOR	Notify PC or on-call supervisor within 15 minutes of conclusion.	Do not wait until end of shift. Every restraint is a Category



		1 CI.
10. COMPLETE DOCUMENTATION	Complete the Physical Restraint Documentation Form (Appendix B) before end of shift.	Completed by the staff member who initiated the restraint; reviewed by shift lead; submitted to PC same shift.

17. RESTRAINT DOCUMENTATION AND REPORTING

17.1 Mandatory Documentation — Same Shift

The Physical Restraint Documentation Form (Appendix B) must be completed before the end of the shift in which the restraint occurred. No exceptions. Required information:

- Youth's name; date; time restraint initiated; time restraint released; duration in minutes; location
- Names of all staff present; names of all other youth present
- Behaviour observed immediately before restraint — specific description, not interpretation
- All de-escalation strategies attempted and their outcome; verbal directions given — exact words
- NVCI hold(s) used — named specifically; confirmation all holds are authorized
- Youth's physical and emotional condition during and after restraint; any injuries; medical assessment
- Post-restraint debrief status; staff member's NVCI certification expiry date

17.2 Reporting Chain

Report	Timeline and Recipient
Verbal report to PC or on-call supervisor	Within 15 minutes of restraint conclusion
PC verbal report to PM	Within 2 hours
PM notification to MCFD social worker or PCS (1-800-663-9122)	Within 2 hours — all restraints are Category 1 CIs requiring same-day MCFD notification
PM notification to ED	Same day
CI Report submitted to MCFD	Within 24 hours
PM personal review and co-	Within 24 hours



signature of restraint documentation	
Post-restraint debrief with youth	Within 24 hours when the youth is regulated
Post-restraint debrief with staff team	Within 48 hours
BSP review triggered	Within 5 business days of every restraint

18. POST-RESTRAINT DEBRIEF AND REVIEW

18.1 Debrief With the Youth

Every restraint is followed by a supported, non-punitive debrief with the youth — when the youth is regulated and willing, within 24 hours. The debrief is not a lecture or a consequence. It is a therapeutic conversation aimed at repair, understanding, and planning.

Debrief Component	Questions to Explore
How are you doing?	Check in on physical and emotional wellbeing. Acknowledge that the experience was hard.
What happened from your perspective?	Hear the youth's experience without interrupting or defending. Document what they say.
What were you feeling?	Validate the feeling even if the behaviour was not okay.
What would have helped?	Genuinely ask what the youth needed and did not get. This is the most important question for BSP improvement.
What happens next?	Confirm the plan; confirm the relationship is intact; confirm any follow-up.
Document and act	Document using Appendix C; use the youth's input to update the BSP within 5 business days

18.2 Staff Debrief

Debrief Focus	Content
What happened?	Factual chronological review — not judgment. What did each person see, hear, and do?
What worked?	Genuine recognition of effective de-escalation and safe technique



What could be different?	Not 'what went wrong' — but 'what do we need more of?' Training; staffing; BSP update
Staff wellbeing	How are staff doing? Restraints are distressing. EFAP actively offered.
Action items	What will be done differently? Named owner and target date. Documented and tracked.

19. RESTRAINT TREND MONITORING AND REDUCTION PLAN

19.1 Restraint Register

The PC maintains a Restraint Register for each program — a log of every restraint with: date; youth; duration; staff; holds used; injuries; MCFD notification confirmed. PC reviews monthly and reports to PM by the 20th of each month.

19.2 Threshold Review Triggers

Threshold	Review Required	Participants
3 or more restraints involving the same youth in 30 days	BSP Threshold Review — Is the current plan effective? What more is needed?	PM, PC, MCFD social worker, youth where appropriate
Any restraint using a prohibited hold	Immediate investigation; staff training review; MCFD notification	PM, ED, HR
Any injury to youth or staff during restraint	Incident review and safety analysis; JHSC notified (OHS component)	PM, ED, JHSC
Year-over-year increase in restraint frequency	Program-level behaviour support review	ED, PM, QA, MCFD Contract Manager

19.3 Restraint Reduction Goal

Evergreen Community Care's organizational goal is a continuous year-over-year reduction in the number and duration of physical restraints across all programs. This is a key performance indicator in the QA Program. A program with zero restraints over 12 months is an organizational milestone recognized in all-staff communications.

PART 6 — Crisis Response

20. CRISIS LEVELS AND RECOGNITION

Recognizing where a youth is in the escalation cycle is the foundation of effective crisis prevention. The following framework is based on the Crisis Development Model from NVC training. Every BSP includes youth-specific indicators for each level.

Level	Description	General Signs	Staff Response Goal
Level 1 — Anxiety	Marked change from baseline; tension, restlessness, or withdrawal	Pacing; fidgeting; quieter than usual; checking the environment; physical tension visible in face or posture	SUPPORTIVE: Reduce stimulation; increase connection; offer genuine choice; do not increase demands
Level 2 — Defensive	Beginning to lose rational thinking; questioning; argumentative	Verbal challenges; testing limits; raised voice; refusing direction; beginning to use profanity	DIRECTIVE + DE-ESCALATE: Clear, calm, simple direction; limit demands; do not argue; call for backup NOW
Level 3 — Acting Out	Loss of rational control; potential for physical aggression	Throwing objects; hitting; kicking; self-harm; running; screaming; physical aggression toward staff or peers	SAFETY FIRST: Clear others; de-escalate verbally; restraint only if all 4 threshold conditions met and NVC current; call 911 if weapons or serious harm occurring
Level 4 — Tension Reduction	Stress response winding down; physically and emotionally exhausted	Crying; sitting quietly; apologizing; seeking reassurance; exhausted appearance; may have no memory of Level 3	RECONNECT: Non-judgmental presence; offer water; no lecture or debrief now; post-crisis procedures begin

21. CRISIS INTERVENTION PROCEDURES BY LEVEL

21.1 Levels 1 and 2 — Preventive and Early Intervention

- Approach calmly and without urgency — your state regulates theirs
- Reduce environmental stimulation — lower noise, reduce people in the space, offer a quiet area
- Make genuine, non-intrusive contact: 'Hey, I notice you seem like you're having a hard time. I'm here.'
- Offer a limited genuine choice — two real options; both acceptable to you
- Remove unnecessary demands — now is not the time for compliance with minor directives
- Call for backup before the situation escalates further — the presence of a second calm adult almost always changes the trajectory



21.2 Level 3 — Active Crisis

- Backup must be present or called immediately
- Clear the area — move other youth and non-essential staff away from the immediate area
- Provide simple, calm, single directives — the youth cannot process complex instructions in this state
- Maintain your own regulation — loud, escalated staff escalate crises
- Call 911 if: weapons are involved; serious physical harm is occurring; restraint is not sufficient to prevent serious harm
- Physical restraint only if all four legal threshold conditions are simultaneously met (Section 14.1)

21.3 Level 4 — Tension Reduction

- Do not immediately debrief or address the behaviour — the youth is not yet regulated enough to benefit
- Offer non-judgmental presence — sit nearby without demanding interaction
- Offer water, a blanket, a quiet space; confirm physical safety — any injury requires medical attention
- Notify supervisor; begin documentation; therapeutic debrief within 24 hours when regulated

22. SELF-HARM AND SUICIDAL CRISIS RESPONSE

⚠ Self-harm and suicidal ideation require immediate, compassionate, and competent response. These are health crises — not behaviour management situations. The following procedures are mandatory.

Situation	Immediate Response	Documentation and Follow-Up
Youth discloses suicidal ideation — no immediate danger	Stay with the youth; listen; do not leave them alone; ask directly — asking directly reduces risk; notify supervisor immediately; contact MCFD social worker	Category 1 CI; BSP and Safety Plan immediate review; MCFD within 2 hours; follow Safety Plan in BSP
Youth actively self-harming — non-life-threatening	Stay calm; stay with the youth; focus on the youth not the means; call supervisor; offer first aid once youth is regulated	Category 1 CI; medical assessment; MCFD social worker within 2 hours; BSP review within 5 days
Youth actively self-harming — life-	Call 911 immediately; stay with the youth; provide first aid within your training; keep the	Category 1 CI; MCFD immediate notification; ED



threatening (severe lacerations, overdose, hanging, jumping)	youth as calm as possible; call supervisor and MCFD simultaneously or immediately after 911	notification same day; full CI report within 24 hours
Youth expresses suicidal ideation in context of distress ('I want to die')	Take it seriously every time — never dismiss or normalize; respond as above; document and notify supervisor	Category 2 CI minimum; document exact words and context; supervisor notified; follow up with youth when regulated

Crisis resources posted in every common area and every youth's room: BC Crisis Line 1-800-784-2433 (24/7); Kids Help Phone 1-800-668-6868; Crisis Text Line: text HOME to 741741; Trans Lifeline 1-877-330-6366.

23. PROPERTY DAMAGE AND AGGRESSION RESPONSE

23.1 Property Damage

Property damage does not meet the threshold for physical restraint unless a person is in the direct path of the damage and cannot move to safety. Move people away; use de-escalation at a safe distance; do not grab the youth to stop property damage unless people are in immediate danger. Property damage over \$500 is a Category 2 CI.

23.2 Verbal Aggression

Verbal aggression does not meet the threshold for physical restraint. It is de-escalated using approved strategies. Staff are not required to remain in the direct path of sustained verbal abuse — maintaining a safe distance while remaining available is appropriate. Document all significant verbal aggression.

23.3 Physical Aggression Toward Staff

If a youth is physically assaulting a staff member and the assault cannot be avoided by stepping away, physical restraint may be used to stop the assault if the legal threshold is met. The restraint ends when the assault stops — not when the youth is fully compliant. Any staff member physically harmed receives: immediate safety check and first aid; medical assessment if injury evident; WorkSafeBC Form 7 if medical treatment required; active EFAP offer within 24 hours; post-incident debrief within 48 hours.

24. POST-CRISIS PROCEDURES



Action	Timeline
Ensure youth's physical safety; first aid or medical attention if needed	Immediately
Notify PC or on-call supervisor	Within 15 minutes
Check on other youth in the home — their wellbeing is also a priority	Within 30 minutes
Begin incident documentation	Before end of shift
PC notifies PM	Within 2 hours
PM notifies MCFD if Category 1	Within 2 hours
Complete CI Report	Within 24 hours (Cat 1) or next business day (Cat 2)
Youth debrief — when regulated and willing	Within 24 hours
Staff debrief	Within 48 hours
BSP review if indicated	Within 5 business days for any Cat 1
EFAP actively offered to all involved staff	Within 24 hours; ongoing



PART 7 — Specific Behaviour Categories

25. ELOPEMENT AND MISSING YOUTH

Elopement — leaving the program without authorization — requires a behaviour support response (prevention and post-elopement planning) and a safety response (missing youth protocol). This section addresses the behaviour component. For the full missing youth safety response, see RC Program Manual v4 Section 27.

25.1 Prevention — BSP Requirements

- BSP documents elopement risk: frequency, pattern, triggers, destinations, and times of day
- Proactive relationship — youth who feel connected to their home and to staff are less likely to elope; relationship investment is the primary elopement prevention strategy
- Identify the unmet need that drives elopement for this youth and address it directly
- Youth are involved in developing their own elopement prevention plan: 'What would make it easier for you to stay?'

25.2 Post-Return Protocol

- Welcome the youth back warmly — the return is the most important moment; a warm welcome makes the next return more likely
- Physical health check — any injury, exposure, or concerning activity documented
- Non-punitive debrief when the youth is regulated — focus on safety and understanding, not consequence
- BSP review — every elopement is information about unmet need; update the BSP accordingly

26. SEXUALIZED BEHAVIOURS

Sexualized behaviours are most often indicators of prior sexual abuse or harmful sexual exposure, not wilful misconduct. They require a therapeutic response grounded in safety and dignity, not a punitive one.

- Do not shame or humiliate the youth. Respond calmly and matter-of-factly.
- Ensure immediate safety — if the behaviour involves or risks harm to another youth, separate and ensure both youth are safe
- Mandatory reporting — if the behaviour discloses or reasonably suggests sexual abuse, the CFCSA s.14 mandatory reporting obligation is triggered immediately; report to MCFD even if uncertain



- Document fully and professionally — clinical language; behaviour observed, not interpretations
- All sexualized behaviour involving contact with another youth is a Category 1 CI; other sexualized behaviour is Category 2 minimum
- Ongoing or escalating sexualized behaviour requires specialized clinical assessment; PM requests referral through MCFD social worker

27. SUBSTANCE USE BEHAVIOURS

Substance use reflects complex underlying need — trauma, developmental stage, coping, and sometimes dependence. The response is harm-reduction informed, compassionate, and safety-focused.

Situation	Response	Required Actions
Youth returns under influence of alcohol or drugs	Ensure physical safety; do not punish for being under influence; health assessment if any concern about intoxication; supportive, non-judgmental reception	Document; notify supervisor; MCFD notification per CI protocol; health assessment if indicated
Youth found with substances on program property	Safety first — unknown substances treated as potentially hazardous; secure the substance safely; never administer any substance to a youth; police notification if quantity indicates possible trafficking	Document; notify supervisor; MCFD notification; secure the substance; document chain of custody
Youth discloses active substance dependence	Non-judgmental reception; follow up with MCFD social worker for assessment and support services; BSP updated	MCFD notification; CI documentation if new safety concern; referral requested
Youth using substances as a coping mechanism	Harm reduction approach: acknowledge the coping function; build alternative regulation strategies; connect to substance use support services	BSP addresses substance use pattern; skill-building goals include alternative coping

28. TECHNOLOGY AND SOCIAL MEDIA MISUSE

Technology access is a right in most contexts — not a privilege. Restrictions on technology must be proportionate, individualized, and never used as a blanket consequence.

- Complete removal of phone or device access is not an authorized behaviour management strategy unless the device has been used to facilitate a specific documented safety risk and is authorized in the BSP
- Reading a youth's private messages without authorization is a privacy violation
- Access to the RCY and crisis support lines is always available regardless of any technology restriction



- Where technology restriction is included in a BSP, it is: time-limited; directly related to a documented safety concern; the minimum restriction necessary; reviewed at every BSP review
- Cyberbullying — perpetration or victimization — is documented as a behaviour concern and addressed in the BSP; MCFD social worker notified

29. PEER-TO-PEER CONFLICT AND AGGRESSION

Situation	Response
Verbal conflict between youth — not escalating	Staff monitor; do not intervene to resolve every peer conflict; facilitate youth's own resolution skills where safe; separate if it becomes personal, sustained, or escalating
Verbal conflict escalating toward aggression	Separate the youth; de-escalate for each youth individually; do not require immediate conflict resolution; ensure safety first
Physical aggression between youth	Separate immediately; ensure both youth are safe; call for backup; verbal de-escalation; 911 if serious harm occurring or imminent; do not place yourself between two youth in a manner that puts you at risk of injury
One youth harming another — bullying, sustained aggression	Category 1 or 2 CI depending on severity; MCFD social workers for both youth notified; BSPs reviewed; consider whether both youth can safely remain in the same program
Sexual contact between youth	Immediate separation; safety check for both; Category 1 CI; MCFD notification; mandatory reporting assessment; both youth need therapeutic support

- Never impose group consequences for the behaviour of one youth — prohibited practice
- Never use peer pressure or group dynamics as a behaviour management strategy
- Positive peer culture — explicitly fostered through programming — is one of the most powerful protective factors in group care



PART 8 — Oversight, Quality Assurance, and Training

30. SUPERVISION AND STAFF SUPPORT

Effective behaviour support depends on well-supported staff. Staff who work with traumatized youth are at high risk of secondary traumatic stress, compassion fatigue, and burnout — all of which reduce the quality of behaviour support and increase the risk of prohibited practices. Supervision and staff support are themselves behaviour support interventions.

30.1 Required Supervision Content

- Review of any behaviour incidents since the last session — what happened, what worked, what was difficult
- Staff's emotional response to challenging behaviours — normalized, not judged
- BSP implementation consistency — are all staff implementing the plan?
- Secondary traumatic stress check-in — EFAP actively offered
- Gender-affirming care practice (MCFD Standard G.4.3(c) — mandatory in every session since April 2024)
- Training needs — what does this staff member need more of to do this work well?

30.2 Post-Incident Support for Staff

- Verbal check-in before leaving the shift following any significant behaviour incident
- Active EFAP offer within 24 hours — confidential; PC or PM does not know if the staff member uses it
- Post-incident debrief within 48 hours — not disciplinary; focused on wellbeing and learning
- Modified duty option if the staff member is experiencing secondary traumatic stress symptoms that affect their capacity to work safely

31. BEHAVIOUR SUPPORT TRAINING REQUIREMENTS

Training	Who	Initial Deadline	Renewal
Non-Violent Crisis Intervention (NVCI) — Full Certification	All direct-care staff; PC	Module 1 before first solo shift; full certification within 90 days	Annual recertification — no exceptions; expired NVCI = no restraint authority
Trauma-Informed Practice	All direct-care staff; PC; PM	Within 60 days of hire	Every 2 years
BSP Development and Implementation	PC; PM; senior direct-care staff	Before first BSP responsibility	Annual



This Behaviour Support Manual — read and sign Appendix H	All staff; students; volunteers	Before first shift with youth	Upon each annual update
Gender-Affirming Care (MCFD Standard G.3.2(b), amended April 2024)	All staff; PC; PM	Within 60 days of hire	Annual — Trans Care BC / PHSA module
Indigenous Cultural Safety and Humility (San'yas or FNHA)	All staff; PC; PM	Within 60 days of hire	Annual
Suicide and Self-Harm Response (ASIST or SafeTALK)	All direct-care staff; PC	Within 60 days of hire	ASIST: biennial; SafeTALK: annual
Prohibited Practices — annual team review	All direct-care staff	At orientation	Annual; also following any prohibited practice concern
Abuse Recognition and Prevention	All staff	Within 30 days	Annual
Mandatory Reporting (CFCSA s.14)	All staff	Within 30 days	Every 2 years

⚠ NVCI certification must be current before any staff member performs physical restraint. An expired NVCI certification means the staff member is not authorized to use restraint — regardless of the situation. PC confirms NVCI expiry dates for all scheduled staff by the 5th of each month.

32. QA MONITORING — BEHAVIOUR SUPPORT INDICATORS

Indicator	Target	Frequency
Physical restraints — total and per youth	Decreasing year-over-year; zero preferred	Monthly PC report; quarterly PM trend analysis
Duration of restraints — average and maximum	Decreasing trend	Monthly
Restraints using any prohibited technique	Zero — zero tolerance	Monthly; immediate report on any occurrence
Restraint-free months across all programs	Increasing	Monthly; recognized in all-staff communications
Staff with current NVCI certification	100%	Monthly by 5th; PC confirms



BSPs completed for all youth within required timelines	100%	Bi-annual file audit
BSP reviews completed on 90-day schedule	100%	Bi-annual file audit
Youth post-restraint debriefs completed within 24 hours	100%	Monthly from restraint register
Staff post-incident debriefs completed within 48 hours	100%	Monthly from supervisor log
Confirmed prohibited practice incidents	Zero — zero tolerance	Immediate; reported to ED and MCFD
Youth satisfaction with behaviour support approach	Positive and improving	Annual survey
Category 1 CIs with behaviour as primary factor	Decreasing trend	Monthly CI log; quarterly trend analysis

33. INCIDENT REVIEW, ROOT CAUSE, AND SYSTEM CHANGE

33.1 Root Cause Analysis Framework

Level of Analysis	Questions
Individual — the youth	What was this youth communicating? What was the unmet need? Does the BSP address it adequately? What does this youth need more of?
Individual — the staff	Did the staff have the training and skills needed? Were they well-supported? Were they in secondary traumatic stress? Is there a coaching or support need?
Team — the shift	Was the BSP being implemented consistently? Was there adequate communication at handover? Was staffing adequate?
Program — the environment	Is the physical environment contributing? Is the program schedule conducive to regulation? Are peer dynamics a factor?
System — the organization	Are there policy gaps? Training gaps? Resource gaps? Are there systemic patterns this incident reflects?

33.2 Corrective Action and Follow-Through



- Every incident review generates a corrective action — or a documented rationale for why no change is needed
- Written with a specific action, named owner, and target date
- Tracked in the Corrective Action Log (QA Program Binder) and followed up at the next supervision session and team meeting
- Closed only when the action is completed and verified — not when it is assigned



PART 9 — Appendices



APPENDIX Appendix A — BEHAVIOUR SUPPORT PLAN TEMPLATE

Complete all fields. No blank fields. Use N/A only where the field genuinely does not apply and document why. PM reviews and approves before MCFD submission.

YOUTH INFORMATION	
Legal Name	
Preferred Name and Pronouns	
Date of Birth	
Cultural / Nation Affiliation	
Program Name	
Room Number	
Program Coordinator	
Program Manager	
MCFD Social Worker and Contact	
Date of This BSP	
BSP Review Date (maximum 90 days)	

SECTION 1 — YOUTH STRENGTHS AND INTERESTS (complete first — minimum 5 genuine strengths)	
Strengths — what this youth does well, what they are proud of	
Interests — activities, topics, relationships the youth genuinely enjoys	
Important relationships — people who matter to this youth	



Cultural connections and sources of identity and pride	
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SECTION 2 — BACKGROUND AND BEHAVIOUR HISTORY	
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Relevant trauma history (broad terms; do not document graphic detail here)	
--	--

Prior diagnoses relevant to behaviour support (if known)	
--	--

Prior BSPs — what worked; what did not	
--	--

Known ineffective responses from prior placements or settings	
---	--

SECTION 3 — CURRENT BEHAVIOUR PATTERNS	
---	--

Specific behaviours this BSP addresses (clinical description)	
---	--

Frequency, Intensity, Duration (FID) baseline for each behaviour	
--	--

ABC Analysis: Antecedent — Behaviour — Consequence pattern	
--	--

Function of behaviour — what need is this behaviour communicating?	
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SECTION 4 — TRIGGERS AND WARNING SIGNS	
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Known antecedents and	
-----------------------	--



triggers (specific to this youth)	
Level 1 (Anxiety) — early warning signs specific to this youth	
Level 2 (Defensive) — escalation signs specific to this youth	
Level 3 (Acting Out) — crisis indicators specific to this youth	
Level 4 (Tension Reduction) — what post-crisis looks like for this youth	

SECTION 5 — DE-ESCALATION STRATEGIES — INDIVIDUALIZED	
Strategies that WORK for this youth (from youth's own input and staff observation)	
Strategies that DO NOT WORK — staff must avoid these	
Sensory supports that help (movement, music, quiet, texture, outdoor, etc.)	
Staff approach and tone that helps this youth	
Environmental modifications that reduce escalation for this youth	

SECTION 6 — APPROVED BEHAVIOUR SUPPORT STRATEGIES	
Positive reinforcement	



strategies authorized for this youth	
Structural and environmental supports	
Logical consequences — pre-approved and proportionate	
Skill-building strategies — what capacity is being built	

SECTION 7 — CRISIS RESPONSE PLAN	
Level 1 response plan specific to this youth	
Level 2 response plan specific to this youth	
Level 3 response plan specific to this youth	
When to call 911 — specific indicators for this youth	
When to contact MCFD during or after crisis	
Language or approach that helps this youth in crisis	
Language or approach to AVOID with this youth in crisis	

SECTION 8 — PHYSICAL RESTRAINT AUTHORIZATION	
Is physical restraint authorized as an emergency measure for this youth? (yes / no)	
Authorized NVCI holds for	



this youth (list specifically)	
Holds CONTRAINDICATED for this youth (medical or trauma history)	
MCFD social worker authorization obtained? (if required by MCFD Policy 5.13)	
Conditions or limitations on restraint use for this youth	

SECTION 9 — POST-INCIDENT PROTOCOL	
What this youth needs immediately after a crisis	
Preferred debrief approach for this youth	
Staff most trusted by this youth for post-incident support	
Sensory and emotional regulation supports post-crisis	

SECTION 10 — SKILL-BUILDING GOAL	
Goal statement (measurable)	
Timeline for goal	
Measurement method — how will we know the goal is being achieved?	

SECTION 11 — CONSENT AND PARTICIPATION	
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Youth's signature or documented verbal participation	
Youth's areas of agreement with the BSP	
Youth's areas of disagreement or concern	
Parent / guardian signature (if applicable)	
MCFD social worker acknowledgement and date	
PC signature and date	
PM approval signature and date	

REVIEW HISTORY	Date	Summary of Changes / Reason for Review
Review 1		
Review 2		
Review 3		



**APPENDIX Appendix B — PHYSICAL RESTRAINT DOCUMENTATION FORM**

⚠ Complete before end of shift. PM co-signs within 24 hours. This form is a Critical Incident document — do not alter after completion.

Youth Legal Name	
Date of Incident	
Time Restraint Initiated	
Time Restraint Released	
Duration (minutes)	
Location of Incident	
Staff Member Initiating Restraint — name and NVCI expiry date	
Second Staff Member Present — name and NVCI expiry date	
Other Youth Present (names)	
NVCI Hold(s) Used — name specifically	
Were all holds used authorized NVCI holds? (yes / no — if No, describe deviation)	
Behaviour observed immediately before restraint — objective and specific	
De-escalation strategies attempted before restraint — list all	
Verbal direction given immediately before restraint — exact words used	
Youth's physical condition during restraint (breathing, colour, responsiveness)	



Youth's condition at time of release	
Injuries to youth — describe; photograph if possible	
Injuries to staff — describe	
Medical assessment provided or offered — detail	
Supervisor notified — name and time	
MCFD notified — name, time, and outcome	
Post-restraint debrief with youth completed? (yes / no; if no, reason and scheduled time)	
Staff member signature and date	
PC review signature and date	
PM co-signature and date (within 24 hours)	



**APPENDIX Appendix C — POST-RESTRAINT DEBRIEF RECORD**

Youth Name	
Date of Original Incident	
Date of Youth Debrief	
Date of Staff Debrief	
Staff Conducting Youth Debrief	
Staff Present at Staff Debrief	
Youth's Account of What Happened	
Youth's Emotional Response to the Incident	
Youth's Response to the Restraint Itself	
What the Youth Said Would Have Helped	
Youth's Physical Wellbeing at Time of Debrief	
Staff Debrief — What Worked	
Staff Debrief — What Would Be Different	
Staff Debrief — Wellbeing of Involved Staff	
EFAP Offered to All Involved Staff? (yes / no)	
BSP Review Required? (yes / no)	
BSP Review Scheduled (date)	
Action Items from Debrief (owner; target date)	
PC Signature and Date	





**APPENDIX Appendix D — CRISIS RESPONSE QUICK REFERENCE CARD**

Post in every staff area and shift communication binder. Does not replace the full manual — shift-accessible quick reference only.

CRISIS LEVEL — QUICK RECOGNITION	FIRST RESPONSE
Level 1 — Anxiety: marked change from baseline; tension; withdrawal	SUPPORT: Reduce stimulation. Increase connection. Offer genuine choice. Do not increase demands.
Level 2 — Defensive: argumentative; testing limits; raised voice; refuses direction	DIRECTIVE + DE-ESCALATE: Calm and clear. Single directions. Remove unnecessary demands. CALL BACKUP NOW.
Level 3 — Acting Out: physical aggression; self-harm; throwing; running	SAFETY FIRST: Clear others. De-escalate verbally. Restraint only if all 4 threshold conditions met and NVCI current. Call 911 if weapons or serious harm occurring.
Level 4 — Tension Reduction: exhausted; crying; apologizing; seeking reassurance	RECONNECT: Non-judgmental presence. Water and quiet. No lecture now. Medical check if any injury.

EMERGENCY CONTACTS	
911 — any life-threatening emergency; weapons; serious harm occurring	911
MCFD Provincial Centralized Screening (24/7)	1-800-663-9122
On-Call Supervisor	[See posted weekly on-call schedule]
BC Crisis Line	1-800-784-2433
Kids Help Phone	1-800-668-6868
Trans Lifeline	1-877-330-6366
Representative for Children and Youth (RCY)	1-800-476-3933

PHYSICAL RESTRAINT —



USE ONLY WHEN ALL FOUR CONDITIONS ARE SIMULTANEOUSLY MET	
1. Imminent serious physical harm to a person is about to occur	Yes / No
2. De-escalation has been tried and is not working in this moment	Yes / No
3. You have current (non-expired) NVCI certification	Yes / No
4. You are using the least restrictive authorized NVCI hold	Yes / No
If any condition is NOT met: do NOT use restraint. Call backup. Call 911 if needed.	

PROHIBITED — NEVER, REGARDLESS OF CIRCUMSTANCES OR WHO TELLS YOU TO	
Prone (face-down) restraint Any pressure on chest / neck / airway / head	ABSOLUTELY PROHIBITED
Corporal punishment Seclusion Withholding meals Humiliation Threats	ABSOLUTELY PROHIBITED
If a supervisor tells you to use a prohibited practice: REFUSE. Document it. Report it.	





APPENDIX Appendix E — DE-ESCALATION STRATEGIES REFERENCE SHEET

Document which of these are effective for each youth in BSP Section 5. Generic de-escalation is a starting point — individualized de-escalation is what actually works.

Category	Strategy	Notes
Environment	Reduce noise — music off, TV off, voices quieter	Reduces sensory overload; often one of the most effective early interventions
Environment	Reduce number of people in immediate area	Creates psychological and physical space; reduces audience effect
Environment	Offer access to a calm, private space (not seclusion — youth chooses voluntarily)	Youth-identified and voluntary; never imposed
Environment	Adjust lighting — reduce bright or harsh lights	Useful for youth with sensory sensitivities; document in BSP if effective
Relationship	Genuine, non-intrusive check-in: 'I'm here. I can see this is hard.'	Connection without pressure
Relationship	Use preferred name and correct pronouns — every time	Affirmation is de-escalating; misgendering is triggering
Relationship	Involve the staff member the youth trusts most	Relationship is the most powerful de-escalation tool
Communication	Lower your voice — don't match the youth's volume	Your regulation regulates theirs
Communication	Reduce sentence length — one idea at a time	Dysregulated brains cannot process complex language
Communication	Name the feeling before addressing the behaviour	'I can see you're really angry' before 'I need you to step back'
Communication	Offer a genuine limited choice	Restores a sense of control; both options must be truly acceptable to you
Communication	Remove demands — reduce expectations in the moment	Immediate demands escalate; fewer demands de-escalate
Communication	Use future-oriented language	'When you're ready, we can...' rather than 'You need



		to right now'
Physical	Get to the youth's level — don't stand over them	Non-threatening posture reduces perceived threat
Physical	Give more personal space than usual	Proxemics matter; closer is often more threatening
Physical	Walk side-by-side if movement helps	Parallel movement is less confrontational than face-to-face
Physical	Offer movement or sensory outlet: 'Do you want to go for a walk?'	Discharges the fight-or-flight physiological state
Self-regulation	Offer a known regulation tool: fidget, blanket, music, journal	BSP documents which tools work for this youth
Self-regulation	Model your own regulation — 'Let's both take a minute'	Explicit modelling of regulation
Strategic withdrawal	Maintain visual contact from a greater distance	Appropriate when proximity is escalating; document reasoning
Strategic withdrawal	Brief, calm exit with follow-up promise: 'I'm going to give you some space. I'll check back in five minutes.'	Not abandonment; planned, brief, with return





APPENDIX Appendix F — PROHIBITED PRACTICES POSTER — STAFF REFERENCE

Post in the staff area of every program. Reviewed at minimum annually with the full team and at every new staff orientation.

THESE PRACTICES ARE PROHIBITED AT ALL TIMES

Regardless of the behaviour. Regardless of any instruction. No exceptions.

X Corporal punishment — any physical contact intended to cause pain or fear as a consequence	CFCSA s.70; RCR s.44
X Prone restraint (face-down) or any pressure on chest, neck, or head	MCFD Policy 5.13 — risk of death
X Seclusion — locking or confining a youth against their will	RCR s.44; MCFD SHSS D.7
X Withholding meals, snacks, or basic care as a consequence	CFCSA s.70; RCR s.44
X Withholding family contact or cultural connection as a consequence	CFCSA s.70
X Humiliation, public shaming, degrading language, or threats	CFCSA s.70; MCFD SHSS D.7
X Withholding prescribed medication as a consequence	RCR s.45-49
X Denying school attendance as a consequence	School Act; MCFD SHSS D.5
X Body or strip searches of any youth	CFCSA s.70
X Blocking access to RCY (1-800-476-3933), Ombudsperson, or police	CFCSA s.70; Advocate for Children and Youth Act
X Financial consequences not authorized in the BSP	CFCSA s.70



X Peer-administered consequences or group punishment for one youth's behaviour	MCFD SHSS D.7
X Recording or photographing youth without MCFD authorization	FOIPPA; CFCSA s.70
X Physical restraint by staff member with expired or no NVCI certification	MCFD Policy 5.13

If you witness a prohibited practice: STOP IT. Ensure youth safety. Report immediately to your supervisor. You are protected from retaliation for reporting in good faith.





APPENDIX Appendix G — BSP COMMUNICATION AND HANDOVER CHECKLIST

Complete for every youth before any staff member's first solo shift with that youth. File in the youth's communication binder. Re-sign whenever the BSP is revised.

Youth Name (legal and preferred)	
BSP Date (version the staff member received)	
Staff Member Name	
Position (permanent / casual / relief / student / volunteer)	
Date of BSP Orientation	
PC Who Conducted BSP Orientation	

I have read and understood the current BSP for this youth, including:	Initials
Section 1: Youth's strengths, interests, and important relationships	
Section 4: This youth's specific triggers and warning signs at each crisis level	
Section 5: De-escalation strategies that WORK and strategies to AVOID for this youth	
Section 6: Approved behaviour support strategies and pre-approved logical consequences	
Section 7: Crisis response plan for this youth	
Section 8: Physical restraint authorization status and any	



contraindicated holds	
Section 9: What this youth needs post-crisis and preferred debrief approach	
I understand that the prohibited practices list (Appendix F) applies without exception	
I know how to reach the on-call supervisor and MCFD PCS (1-800-663-9122)	
I know the location of this youth's current BSP in the shift binder	
My NVCI certification is current — expiry date:	

Staff Member Signature	
Date	
PC Co-Signature (confirms BSP orientation completed)	
Date	





APPENDIX Appendix H — BEHAVIOUR SUPPORT STAFF ACKNOWLEDGEMENT FORM

All staff, students, and volunteers must read and sign this acknowledgement before their first shift with youth and upon each annual update of this manual.

I acknowledge that I have received, read, and understood the Evergreen Community Care Behaviour Support Policy and Procedure Manual (ECC-BSP-2026-001), Version 1.0, effective July 1, 2026.	
I understand that this manual governs all behaviour support practices at every Evergreen Community Care residential care program.	
I understand that the prohibited practices listed in Section 12 and Appendix F are prohibited absolutely, regardless of the behaviour of any youth, regardless of any instruction from any person including a supervisor, and regardless of any claimed emergency.	
I understand that physical restraint may only be used when all four legal threshold conditions in Section 14.1 are simultaneously met, and only when I hold a current, non-expired NVCI certification.	
I understand that prone restraint and any pressure on a youth's chest, neck, or airway are absolutely prohibited and carry a risk of death.	
I understand that every child and youth in care retains all rights under CFCSA s.70 at	



all times, and that no right may be suspended or withheld as a behaviour consequence.	
I understand my personal, non-delegatable mandatory reporting obligation under CFCSA s.14 and my obligation to report any suspected abuse or prohibited practice to MCFD at 1-800-663-9122 regardless of any instruction from my supervisor.	
I understand that gender-affirming care (MCFD Standard G.3.2(b), amended April 2024) is mandatory — I will use every youth's correct name and pronouns consistently, and I understand that deliberate misgendering is a Code of Ethics violation.	
I understand that Indigenous cultural identity, practices, and connection are never restricted as behaviour consequences and are actively facilitated.	
Questions about this manual are directed to my Program Coordinator or Program Manager.	

Staff / Student / Volunteer Name (print)	
Position	
Program	
Signature	
Date	
PC Signature (confirms	



receipt and orientation)	
Date	

Version History

Version	Date	Summary of Changes
1.0	July 1, 2026	Initial standalone Behaviour Support Policy and Procedure Manual; consolidates and significantly expands all behaviour support content previously distributed across the RC Program Manual, PC Manual, PM Manual, and OHS Program Binder; 33 sections in 9 Parts; 8 Appendices including BSP template, restraint documentation form, post-debrief record, crisis quick reference card, de-escalation strategies reference, prohibited practices poster, BSP handover checklist, and staff acknowledgement form; aligned to MCFD SHSS Standards C, D, G (April 2026); gender-affirming care amendments April 2024; MCFD Policy 5.13 (physical restraint); CFCSA s.70 rights framework; DRIPA; OHS Reg. s.4.27

Signatory	Signature / Date
Executive Director	Signature: _____ Date: _____